

What does Operation Fistula do?

Operation Fistula exists to end obstetric fistula for every woman everywhere.

Obstetric fistula is the physical manifestation of entrenched societal and institutional inequity. Its persistence is a direct result of systems failing to serve women and girls. Obstetric fistula primarily affects women in remote areas of the world, who tend to be isolated, and socially ostracized as a result of their injury. All of this contributes to a poor level of understanding of the condition, and a lack of data on its prevalence and incidence globally.

Operation Fistula works to solve three problems:

1. That funding doesn't reach the people who need it.
2. Organizations don't collaborate well.
3. Fistula is seen as a low priority symptom.

We address these problems by:

1. Providing individual performance based funding:

- This funding is driven by the data-collection of our partners.
- We started by funding surgeons alone - having them digitize their patient records - but have now expanded across the continuum of care.

2. Using tech to improve quality and bring everyone together:

- We've assembled a suite of technology in partnership with CommCare, Exasol, Alteryx, and Tableau.

3. Leveraging data on fistula to map gender inequality

- The technology and data gives us an unprecedented view into how systems are failing women and girls. Collecting data on the myriad of indicators that contribute to the persistence of fistula enables us to better target our work, connect with other advocates for gender equality, and deepen the understanding of fistula globally.

The Dataset

<https://datacatalog.worldbank.org/dataset/world-development-indicators>

"The primary World Bank collection of development indicators, compiled from officially-recognized international sources. It presents the most current and accurate global development data available, and includes national, regional and global estimates"

The World Bank collection of development indicators reports on hundreds of indicators. We have selected a handful of these that we think correlate to the prevalence of obstetric fistula.

Obstetric fistula is a condition that tends to occur in places where gender inequality is at its most extreme. Knowing about these related factors, and increasing awareness around them, may reveal new insights and opportunities for us to strengthen our work to end fistula.

There are currently no clear estimates on the prevalence of obstetric fistula around the world, and in different countries. However, we do know which countries host facilities that treat women with obstetric fistula, because we have access to the Global Fistula Map:

<https://www.globalfistulamap.org/>. Based on this information, we have added a field to this dataset that indicates if obstetric fistula is a known issue in a country or not.

We are interested in learning more about the possible correlations between the different metrics we have selected, or get a clearer picture of just one of these factors. Ideally, in relation to the indicator if this is a fistula country or not.

Data dictionary

Field	Definition
Country	Country that indicator relates to.
Year	Year that indicator relates to.
Births attended by skilled health staff (% of total)	Births attended by skilled health staff are the percentage of deliveries attended by personnel trained to give the necessary supervision, care, and advice to women during pregnancy, labor, and the postpartum period; to conduct deliveries on their own; and to care for newborns. The share of births attended by skilled health staff is an indicator of a health system's ability to provide adequate care for pregnant women.
Current health expenditure (% of GDP)	Level of current health expenditure expressed as a percentage of GDP. Estimates of current health expenditures include healthcare goods and services consumed during each year. This indicator does not include capital health expenditures such as buildings, machinery, IT and stocks of vaccines for emergency or outbreaks. The health expenditure estimates have been prepared by the World Health Organization under the framework of the System of Health Accounts 2011 (SHA 2011). The Health SHA 2011 tracks all health spending in a given country over a defined period of time regardless of the entity or institution that financed and managed that spending. The levels and trends of health expenditure data identify key issues such as weaknesses and strengths and areas that need investment, for instance additional health facilities, better health information systems, or better trained human resources.
Current health expenditure per capita (current US\$)	Current expenditures on health per capita in current US dollars. Estimates of current health expenditures include healthcare goods and services consumed during each year. The levels and trends of health expenditure data identify key issues such as weaknesses and strengths and areas that need investment, for instance additional health facilities, better health information systems, or better trained human resources.
Female genital mutilation prevalence (%)	Percentage of women aged 15–49 who have gone through partial or total removal of the female external genitalia or other injury to the female genital organs for cultural or other non-therapeutic reasons.
GDP (current US\$)	GDP at purchaser's prices is the sum of gross value added by all

	resident producers in the economy plus any product taxes and minus any subsidies not included in the value of the products. It is calculated without making deductions for depreciation of fabricated assets, or for depletion and degradation of natural resources. Data are in current U.S. dollars.
Hospital beds (per 1,000 people)	Hospital beds include inpatient beds available in public, private, general, and specialized hospitals and rehabilitation centers. In most cases beds for both acute and chronic care are included. Health systems - the combined arrangements of institutions and actions whose primary purpose is to promote, restore, or maintain health (World Health Organization, World Health Report 2000) - are increasingly being recognized as key to combating disease and improving the health status of populations. The World Bank's Healthy Development: Strategy for Health, Nutrition, and Population Results emphasizes the need to strengthen health systems, which are weak in many countries, in order to increase the effectiveness of programs aimed at reducing specific diseases and further reduce morbidity and mortality.
Life expectancy at birth, female (years)	Life expectancy at birth indicates the number of years a newborn infant would live if prevailing patterns of mortality at the time of its birth were to stay the same throughout its life. Life expectancy at birth used here is the average number of years a newborn is expected to live if mortality patterns at the time of its birth remain constant in the future. It reflects the overall mortality level of a population, and summarizes the mortality pattern that prevails across all age groups in a given year. It is calculated in a period life table which provides a snapshot of a population's mortality pattern at a given time. It therefore does not reflect the mortality pattern that a person actually experiences during his/her life, which can be calculated in a cohort life table. High mortality in young age groups significantly lowers the life expectancy at birth. But if a person survives his/her childhood of high mortality, he/she may live much longer. For example, in a population with a life expectancy at birth of 50, there may be few people dying at age 50. The life expectancy at birth may be low due to the high childhood mortality so that once a person survives his/her childhood, he/she may live much longer than 50 years.
Life expectancy at birth, male (years)	See above.
Lifetime risk of maternal death (%)	Lifetime risk of maternal death is the probability that a 15-year-old female will die eventually from a maternal cause

	assuming that current levels of fertility and mortality (including maternal mortality) do not change in the future, taking into account competing causes of death. Reproductive health is a state of physical and mental well-being in relation to the reproductive system and its functions and processes. Means of achieving reproductive health include education and services during pregnancy and childbirth, safe and effective contraception, and prevention and treatment of sexually transmitted diseases. Complications of pregnancy and childbirth are the leading cause of death and disability among women of reproductive age in developing countries. Maternal mortality is generally of unknown reliability, as are many other cause-specific mortality indicators. Household surveys such as Demographic and Health Surveys attempt to measure maternal mortality by asking respondents about survivorship of sisters. The main disadvantage of this method is that the estimates of maternal mortality that it produces pertain to any time within the past few years before the survey, making them unsuitable for monitoring recent changes or observing the impact of interventions. In addition, measurement of maternal mortality is subject to many types of errors. Even in high-income countries with reliable vital registration systems, misclassification of maternal deaths has been found to lead to serious underestimation. The estimates are based on an exercise by the Maternal Mortality Estimation Inter-Agency Group (MMEIG) which consists of World Health Organization (WHO), United Nations Children's Fund (UNICEF), World Bank, and United Nations Population Fund (UNFPA), and include country-level time series data. For countries without complete registration data but with other types of data and for countries with no data, maternal mortality is estimated with a regression model using available national maternal mortality data and socioeconomic information. In countries with a high risk of maternal death, many girls die before reaching reproductive age. Lifetime risk of maternal mortality refers to the probability that a 15-year-old girl will eventually die due to a maternal cause.
Nurses and midwives (per 1,000 people)	Nurses and midwives include professional nurses, professional midwives, auxiliary nurses, auxiliary midwives, enrolled nurses, enrolled midwives and other associated personnel, such as dental nurses and primary care nurses. Health systems - the combined arrangements of institutions and actions whose primary purpose is to promote, restore, or maintain health (World Health Organization, World Health Report 2000) - are increasingly being recognized as key to combating disease and improving the health status of populations. The World Bank's Healthy Development: Strategy

	for Health, Nutrition, and Population Results emphasizes the need to strengthen health systems, which are weak in many countries, in order to increase the effectiveness of programs aimed at reducing specific diseases and further reduce morbidity and mortality. To evaluate health systems, the World Health Organization (WHO) has recommended that key components - such as financing, service delivery, workforce, governance, and information - be monitored using several key indicators. The data are a subset of the key indicators. The WHO estimates that at least 2.5 medical staff (physicians, nurses and midwives) per 1,000 people are needed to provide adequate coverage with primary care interventions (WHO, World Health Report 2006).
Proportion of women subjected to physical and/or sexual violence in the last 12 months (% of women age 15-49)	Proportion of women subjected to physical and/or sexual violence in the last 12 months is the percentage of ever partnered women age 15-49 who are subjected to physical violence, sexual violence or both by a current or former intimate partner in the last 12 months.
Teenage mothers (% of women ages 15-19 who have had children or are currently pregnant)	Teenage mothers are the percentage of women ages 15-19 who already have children or are currently pregnant. Having a child during the teenage years limits girls' opportunities for better education, jobs, and income. Pregnancy is more likely to be unintended during the teenage years, and births are more likely to be premature and are associated with greater risks of complications during delivery and of death.
Women who were first married by age 15 (% of women ages 20-24)	Women who were first married by age 15 refers to the percentage of women ages 20-24 who were first married by age 15. Although the legal age of marriage is defined as 18 years in most countries, the practice of child marriage remains widespread. A women's access to education and later her employment opportunities as well as the nature and terms of her work are often compromised by this practice.
Fistula country	There currently exist no clear estimates on the prevalence of OF around the world and in different countries. However, we know in which countries there are facilities that treat women with OF from the Global Fistula Map (https://www.globalfistulamap.org/). Based on this information, we have added a field to this dataset that indicates if OF is a known issue in a country or not.